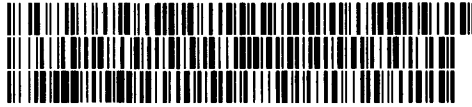


UUID:BB85F844-C710-41D5-8620-E246DC9D2C6A
TCGA-A7-A6VY-01A-PR Redacted



ICD-0-3
Carcinoma, infiltrating NOS
8500/3
Site @ Breast NOS
C50.9
JAD 7/25/13

Final Surgical Pathology Report

Procedure:

Diagnosis

A. Right breast, excisional biopsy:

Infiltrating ductal carcinoma, grade 3, 3.5 cm in greatest dimension.

Invasive carcinoma 1 mm from posterior margin of resection.

Ductal carcinoma in situ, solid subtype, nuclear grade 3, accounting for less than 5% of the tumor, margins uninvolved

B. Right axillary contents, resection:

Metastatic carcinoma in 2 of 14 lymph nodes, size of largest metastasis 3.4 cm, extracapsular extension present (2/14).

Microscopic Description:

Microscopic examination performed.

A. Invasive Carcinoma: present

Histologic type: infiltrating ductal cancer

Histologic grade:

Overall grade: 3

Architectural score: 3

Nuclear score: 3

Mitotic score: 3

Greatest dimension (pT): 3.5 cm, pT2

Specimen margins: negative but close, 1 mm from the posterior margin (black in block A6)

Vessel invasion: not identified

Ductal carcinoma in situ: present

Histologic pattern: solid

Nuclear grade: 3

Central Necrosis: focal

% DCIS of total tumor (if mixed): < 5%

Extensive intraductal component (present/absent): absent

Specimen margins: negative, 3 mm from posterior

Calcification: not identified

B. Sections of the axillary dissection demonstrate metastatic carcinoma in 2 of 14 lymph nodes, (pN1). The size of largest metastasis is 3.4 cm. Extracapsular extension is present.

Specimen

A. Right breast mass

B. Right axillary contents

Clinical Information

Right breast cancer -year-old black female with right cancer and positive nodes

Gross Description

Received fresh in a transpec container, subsequently fixed in formalin labeled "right breast mass" is a 6.0 x 5.5 x 4.5 cm yellow pink fatty tissue fragment which has a short suture designating superior and a long suture designating anterior. The margins are inked as follows based upon the orienting sutures: superior - orange; anterior - blue; posterior - black; inferior - green. The specimen is serially

sectioned from medial to lateral. There is a 3.5 x 3.2 x 2.5 cm ill circumscribed white tan mass centrally located, and within 0.2 cm of the closest margin (deep). White rice-like pellets or gross identified in the central which we be grossly consistent with a nrevious biopsy site. The specimen is received in pathology it nd fixed in formalin and . Representative sections of the specimen are submitted as follows: 1 - medial margin perpendicular, 2 - lateral margin perpendicular. 3 - 10 - representative sections of the remainder of the specimen.

B. Received fresh and subsequently fixed in formalin labeled "right axillary contents" is a 12.0 x 6 x 5 x 2.5 cm aggregate of yellow fatty tissue grossly consistent with axillary contents. The specimen has a suture designating superior. Multiple lymph nodes are palpated the largest measuring 3.4 cm and located midway through the specimen. The highest is markedly indurated as is the largest lymph node. The lymph nodes are submitted from highest to lowest as follows: 1 -lymph node bisected, 2 - 3 possible lymph nodes, 3 - 4 - 3 possible lymph nodes each, 5 - representative section of largest positive lymph node,/1 possible lymph node bisected, 7 - 2 possible lymph nodes, 8 - 1 possible lymph node.

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
IPAA Discrepancy		☒
Prior Malignancy History		☒
Local/Synchronous Primary Noted		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 7/11/13	